

Clinical Assessment Notes

Date of Service: July 10, 2026

Provider: Dr. Evelyn Carter, Orthopedic Surgery

Patient: Arthur Pendelton

Chief Complaint: Severe, unremitting left knee pain.

History of Present Illness:

Mr. Pendelton is a 68-year-old male presenting for an orthopedic consultation regarding his left knee. He reports that the pain has progressively worsened over the last 24 months. He experiences sharp pain with weight-bearing, significant morning stiffness, and requires a cane for ambulation outside the home.

Objective Examination:

Left knee shows a slight varus deformity. Moderate effusion is present. Palpation reveals severe tenderness along the medial joint line.

Range of Motion: 5 degrees extension to 95 degrees flexion with crepitus.

Ligamentous exam: ACL, PCL, MCL, and LCL are stable.

Standardized Assessment:

I administered the WOMAC (Western Ontario and McMaster Universities Osteoarthritis Index) questionnaire today. His standardized joint pain assessment score is 82, confirming severe functional limitation in his daily life.

Assessment & Plan:

Diagnosis of advanced osteoarthritis of the left knee (ICD-10 M17.11).

X-rays confirm Kellgren-Lawrence Grade 4 changes.

Patient has completed a 14-week trial of conservative non-surgical management, including NSAIDs, injections, and physical therapy, which has completely failed to provide relief.

Given the objective imaging, severe functional deficits, and exhaustion of conservative measures, we discussed surgical intervention. The patient wishes to proceed with a Left Total Knee Arthroplasty (CPT 27447). Risks, benefits, and alternatives were discussed in detail. Prior authorization will be initiated.